

HOW TO OPEN AN IN-HOME (PRIVATE CARE) AGENCY in Arkansas

An ADH-licensed Private Care Agency — private pay & Medicaid (Personal Care/ARChoices).

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Arkansas model — read this first

In Arkansas, a business that provides hands-on non-medical personal/attendant care in people's homes is licensed by the Arkansas Department of Health (ADH), Health Facility Services, as a Private Care Agency — under Ark. Code Ann. § 20-10-2301 et seq. and the Rules for Private Care Agencies (effective 2024). A 2025 law (Act 853) moved this license from a DHS certification to direct ADH licensure. If you provide only companion/sitter/homemaker help (no hands-on care) and don't bill Medicaid, you generally don't need the ADH license — but you must still meet worker-training and background-check rules. This guide covers the licensed Private Care Agency path, broken down so a first-timer can follow it.

Name check — it's a “Private Care Agency,” not a “Home Caregiver Agency.” Arkansas doesn't license a “Home Caregiver Agency,” and there's no “Home Caregiver Registry.” The correct ADH license for hands-on non-medical personal/attendant care is the Private Care Agency license (Rules for Private Care Agencies, 2024; Act 853 of 2025). Skilled/medical care is a separate Home Health Agency license. Get the name right when you call ADH.

Budget for the \$1 million liability insurance. Arkansas's Private Care Agency rules require a \$1,000,000 liability insurance policy — a real, specific requirement to plan for (Part 3). A pure companion/sitter, non-Medicaid business may avoid the ADH license, but a hands-on personal-care or Medicaid agency needs the license and the insurance.

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with ADH, Arkansas Medicaid, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed private care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, and the \$1M liability policy (Part 3)
3	Get the ADH license	Application, the pre-license inspection (Part 4)
4	Set up your administrator & RN	Administrator + RN supervision (Part 5)
5	Recruit & screen caregivers	40-hr training, ASP/FBI + registry checks, TB (Part 5)
6	Set up EVV & operations	AuthentiCare EVV, policies, systems (Part 6)
7	Serve private-pay clients	Start earning while Medicaid enrollment proceeds (Part 8)
8	Enroll for Medicaid	Gainwell enrollment + Personal Care/ARChoices (Part 8)
9	Get your first clients	Discharge planners, the 8 AAAs,

		Choices in Living (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Line up your \$1M liability policy and your RN early, build your policies to the Private Care Agency rules, and expect a pre-license inspection. Launch on private pay, then enroll with Arkansas Medicaid (fee-for-service Personal Care and ARChoices) for volume.

PART 1

What This Business Is

In short: An ADH-licensed Private Care Agency — caregivers who provide hands-on personal/attendant care and homemaker help in clients' homes. Agency-based, paid by private pay and Medicaid.

Arkansas licenses Private Care Agencies to provide non-skilled personal/attendant care — hands-on help with the activities of daily living (bathing, dressing, grooming, toileting, transferring, eating, mobility) and household tasks. It's the non-medical home-care license; skilled/medical care is a separate ADH Home Health Agency license.

Private Care Agency vs. skilled Home Health Agency

	Private Care Agency (this guide)	Home Health Agency (skilled)
What it is	Non-skilled personal/attendant care, homemaker	Nursing, therapy under a plan of care
Licensed by	ADH Health Facility Services	ADH (usually Medicare-certified)
Who delivers it	Trained caregivers (RN-supervised)	RNs, LPNs, therapists
Medications	Assistance only	Nurses administer/manage medications

Key terms. ADH = Arkansas Dept. of Health (your licensor). HFS = Health Facility Services. DHS/DAABHS = Dept. of Human Services / Aging, Adult & Behavioral Health Services (Medicaid). Personal Care = the Medicaid state-plan benefit (RN-supervised). ARChoices = the Medicaid HCBS waiver (nursing-facility level of care). PASSE = the managed-care program (only for behavioral-health/IDD — not your population). AuthentiCare = the EVV system for personal care. ASP = Arkansas State Police (background checks). NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Arkansas has one of the fastest-aging, most rural populations in the country, and its low cost of living means a lean home-care agency can reach break-even on a modest census — while the state's fee-for-service Personal Care and ARChoices programs give you a real Medicaid payer alongside private pay. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: ADH licenses and inspects you, DHS/DAABHS and Gainwell handle Medicaid (fee-for-service), the Arkansas State Police and abuse registries screen your staff, and AuthentiCare verifies personal-care visits.

ADH — Health Facility Services (your licensor)

The Arkansas Department of Health, Health Facility Services, issues your Private Care Agency license, writes the Rules for Private Care Agencies, reviews your application, and conducts the pre-license inspection (and inspections at least every 3 years). Phone 501-661-2201; toll-free 1-800-223-0340; adh.hfs@arkansas.gov.

DHS/DAABHS, Gainwell & Medicaid

Arkansas Medicaid personal care runs fee-for-service through DHS's medical services and DAABHS (which administers ARChoices). You enroll as a provider through Gainwell (the fiscal agent). PASSE (managed care) covers only behavioral-health and developmental-disability populations, not aged/physically-disabled personal care. Gainwell Provider Assistance Center: 800-457-4454.

Background checks & registries

Before client contact, each caregiver clears an Arkansas State Police criminal check (with a fingerprint-based FBI check for anyone without 5 years' Arkansas residency), plus checks of the Adult and Long-Term Care Facility Resident Maltreatment Central Registry and the Child Maltreatment Central Registry, and (for Medicaid) the Sex Offender Registry and a drug screen.

EVV — AuthentiCare

For Medicaid personal care (and self-directed IndependentChoices), Arkansas uses AuthentiCare for Electronic Visit Verification (home health uses Sandata). A visit must be verified to be paid. Private-pay work isn't subject to EVV.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Arkansas you form the LLC through the Arkansas Secretary of State — you file Articles of Organization (Form LL-01) online at sos.arkansas.gov, and the filing fee is \$45 online (\$50 by mail). You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization (Form LL-01).** Online at sos.arkansas.gov — the filing fee is \$45 online (\$50 by mail). This creates the LLC.
- 2. Name an Arkansas registered agent.** A person or service with a physical Arkansas street address who receives legal mail — this can be you.
- 3. Adopt an Operating Agreement.** The internal document that sets out ownership, management, and how money moves — Arkansas doesn't file it, but your bank and partners will want it.
- 4. Calendar the annual franchise tax and register with DFA.** Arkansas LLCs pay a \$150 flat annual franchise tax to the Secretary of State, due by May 1 (first due the year after formation). Then register with Arkansas DFA through ATAP (atap.arkansas.gov) for employer withholding and unemployment insurance.

No newspaper publication in Arkansas. Unlike some states, Arkansas has no LLC publication requirement — forming the entity is a straightforward online filing plus your annual \$150 franchise tax.

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it to enroll with Arkansas Medicaid and bill for Personal Care and ARChoices services. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Arkansas branches — Arvest (Arkansas-based), Bank OZK, Simmons Bank, Centennial Bank, or a local credit union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that risk — and Arkansas's Private Care Agency rules go further than most states by requiring a \$1,000,000 liability insurance policy as a condition of the license. Plan for it from the start, alongside the other coverages home care needs:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — \$1M required by the AR rule
Abuse & Molestation	Allegations of abuse by a caregiver — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Surety Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your caregivers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring)	Required in Arkansas for employees

The \$1M policy is a hard license requirement. Arkansas's Private Care Agency rules require a \$1,000,000 liability policy before ADH issues the license — this isn't optional or a nice-to-have, so bind it early and make sure the certificate names the agency exactly as licensed.

Real places to get it

Buy through an independent agent or broker who writes home-care programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Arkansas agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the policy meets the \$1,000,000 liability requirement before you rely on a quote.

What to ask the insurance agent

“Do you write Arkansas Private Care Agencies — which carriers, and what's the minimum premium?”

“Can you meet the \$1,000,000 liability requirement, and is abuse & molestation included or a sub-limit?”

“Do you include non-owned & hired auto and an employee-dishonesty bond?”

“What will Arkansas workers' comp cost given my caregiver payroll?”

PART 4

Get Your ADH License — Step by Step

In short: Assemble your application, policies, and \$1M insurance; submit to ADH Health Facility Services; and pass the pre-license inspection. The license runs on a calendar year and expires December 31.

4.1 Before you apply

- ☐ Entity formed; EIN and NPI obtained; the \$1,000,000 liability policy bound
- ☐ A qualified administrator and an RN for supervision identified (Part 5)
- ☐ Your policies & procedures written to the Rules for Private Care Agencies (client care, caregiver training, background checks, RN supervision, infection control, complaints, incidents)
- ☐ Your Arkansas primary office in place

4.2 The application & inspection

- 1. Submit the ADH application.** On ADH-prescribed forms to Health Facility Services (adh.hfs@arkansas.gov; 501-661-2201).
- 2. Show your \$1M liability insurance** and your policies.
- 3. Pass the pre-license inspection.** ADH conducts a review/inspection before the license is issued; thereafter at least every 3 years.
- 4. Receive your license.** It's issued on a calendar-year basis and expires December 31 — renew annually.

4.3 What the pre-license inspection looks for

The pre-license inspection is a document-and-readiness review, not a mystery — an inspector confirms your paperwork and policies match the Rules for Private Care Agencies. Have these ready and you sail through:

- ☐ Your \$1,000,000 liability certificate, naming the agency as licensed
- ☐ A complete policy & procedure manual (client care, RN supervision, caregiver training, background checks, infection control, complaints, incidents, emergency preparedness)
- ☐ A qualified administrator on record and an RN identified for supervision
- ☐ Personnel files ready to open — background checks, 40-hour training, and competency for each caregiver
- ☐ Client-file templates and your service-agreement and consent forms
- ☐ Your Arkansas primary office set up and reachable

4.4 Timeline

ADH doesn't publish a fixed turnaround, and the license fee isn't stated in the rule — confirm both with Health Facility Services. Submit a complete application, start your Medicaid enrollment and caregiver hiring in parallel, and launch on private pay as soon as you're licensed.

PART 5

Your Administrator, RN & Caregivers — by Position

In short: Arkansas requires an administrator, RN supervision, 40 hours of caregiver training, and layered background checks. Here it is, one role at a time.

5.1 Your administrator & RN

The administrator must be a physician, an RN, or a person with at least one year of supervisory/administrative experience in home health care or a related program (a supervisor must be a licensed nurse or have two years of higher education). A registered nurse designs the individualized service plan and provides supervision — for Medicaid state-plan Personal Care, an RN supervisory visit is required at least annually (more often based on documented risk). Line up your RN early.

5.2 Your caregivers — training & competency

- **Training (ADH Private Care track):** a minimum of 40 hours of training (classroom + clinical), RN-supervised, with the RN observing competency; plus at least 12 hours of in-service each 12 months.
- **Medicaid track:** aides serving Medicaid clients generally hold an active CNA, Home Health Aide, or Certified Personal Care Aide credential (or pass background checks + a drug screen) — confirm the exact DHS training requirement.
- **TB & infection control:** follow current CDC TB guidelines as part of infection control (confirm the specific screening expectation with ADH).

5.3 Background checks — the system you must run

1. **Run the Arkansas State Police criminal check.** Plus a fingerprint-based FBI check for anyone without 5 continuous years of Arkansas residency (ASP fee about \$22–\$24; cbc.ark.org).
2. **Check the maltreatment registries.** The Adult and Long-Term Care Facility Resident Maltreatment Central Registry and the Child Maltreatment Central Registry (re-check at least every 2 years).
3. **For Medicaid, add the Sex Offender Registry and a drug screen.** Repeat criminal checks per the schedule (every 5 years).
4. **Document everything.** Keep dated results in each personnel file — an inspector will ask.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

EVV, Policies & Getting Operational

In short: Write your policies, set up AuthentiCare EVV for Medicaid personal care, and keep files inspection-ready. EVV is the gate to Medicaid payment.

6.1 Your policies & procedures

Build a P&P Manual to the Rules for Private Care Agencies: client rights, intake and the RN service plan, caregiver training and competency, background checks, RN supervision, infection control, complaint handling, incident and abuse reporting, and emergency preparedness. This is your operating backbone and what ADH inspects.

6.2 Set up EVV (before your first Medicaid visit)

Arkansas Medicaid personal care (and self-directed IndependentChoices) uses AuthentiCare for Electronic Visit Verification. Onboard, set up your caregivers/clients/authorizations, capture every visit, and bill against verified visits. Private-pay work isn't subject to EVV.

6.3 Systems

Choose a home-care platform for scheduling, records, invoicing, and payroll that works with AuthentiCare for your Medicaid line.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (privately or via Medicaid), your RN assesses and sets the service plan, a caregiver delivers the hours (with EVV for Medicaid), the RN supervises, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or a DHS/DAABHS case manager (Part 10).

What you do on the first call:

- Take the basics — who needs care, what tasks, how many hours, where, and when to start; confirm who pays (private, Medicaid Personal Care/ARChoices, VA, or long-term-care insurance); and confirm the need is within your scope and service area.

Stage 2 — RN assessment & the service plan

Your RN assesses the client and builds the individualized service plan the caregiver follows. For Medicaid, eligibility and level of care are assessed via the Arkansas Independent Assessment; your plan aligns to the authorization.

What the RN reviews in the home:

- Mobility and transfer needs; fall hazards; medications and who manages them; family and other supports; and, for Medicaid clients, the authorization that sets the approved hours your plan must match.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, trained caregiver and start with RN oversight.

Before the first shift, confirm:

- The caregiver's background checks and 40-hour training are documented and current; the service agreement and consents are signed; and the caregiver has the plan, the schedule, and a way to reach your on-call line.

Stage 4 — Delivering the hours (with EVV for Medicaid)

The caregiver delivers care; Medicaid visits are captured in AuthentiCare. Keep visit notes and protect the client-caregiver match — consistency drives satisfaction and retention.

Stage 5 — RN supervision & quality

Your RN makes supervisory visits (at least annually for Medicaid Personal Care, more often based on risk); you run your quality program and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, the RN updates the service plan; for Medicaid clients, coordinate reassessments and new authorizations. Keep authorizations current so delivered hours get paid.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any payer, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, Medicaid, VA & LTC Insurance

In short: Start on private pay and long-term-care insurance the day you're licensed; add Medicaid through fee-for-service Personal Care and the ARChoices waiver. Arkansas is a low-cost market.

Your four revenue streams:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Start immediately; ~\$26/hr in AR (2024, low-cost)
Medicaid — Personal Care & ARChoices	State-plan Personal Care + the HCBS waiver	Fee-for-service (not PASSE); requires enrollment & EVV
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA, Veteran-Directed Care, Aid & Attendance	Serve veterans via the Community Care Network (Region 3, Optum)

8.1 Private pay — your launch market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. Arkansas is a low-cost market: non-medical care runs about \$26/hour (2024 cost-of-care data — verify the current figure). Set your rate above your fully-loaded caregiver cost. Long-term-care insurance is part of this stream: many policies reimburse in-home personal care once ADL/cognitive triggers are met (most require a licensed agency) — you bill the family or the insurer.

8.2 Medicaid — Personal Care & ARChoices (fee-for-service)

Arkansas Medicaid pays for in-home personal care two main ways: the state-plan Personal Care benefit (an entitlement — RN-supervised, no nursing-facility level of care required) and the ARChoices in Homecare waiver (for people 65+, or 21–64 with a physical disability, at nursing-facility level of care). Both run fee-for-service through DHS/DAABHS — PASSE managed care covers only behavioral-health/IDD populations, not yours.

1. **Hold your ADH license first (Part 4).**
2. **Enroll as an Arkansas Medicaid provider.** Through the Gainwell provider portal (portal.mmis.arkansas.gov) with your NPI; complete EVV enrollment. Gainwell 800-457-4454.
3. **Serve Personal Care and/or ARChoices clients** authorized through DHS assessment/case management. Members may also self-direct via IndependentChoices.
4. **Set up AuthentiCare EVV (Part 6)** before you deliver a Medicaid visit.

Personal care is billed under HCPCS T1019 (per 15 minutes) on the DHS fee schedule — pull the current rate from the live schedule (older posted figures are out of date), and confirm the timely-filing window.

8.3 Veterans

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Arkansas is in Region 3, administered by Optum (844-839-6120). Arkansas VA medical centers: Central Arkansas (Little Rock, 501-257-1000) and the Ozarks (Fayetteville, 479-443-4301).
- **Aid & Attendance:** an increased VA pension the veteran can apply toward your private-pay care (verify current amounts on [VA.gov](https://va.gov)).

8.4 Enrolling with Arkansas Medicaid — Personal Care & ARChoices, walked through

This is the Arkansas-specific enrollment path that trips up first-timers, so here it is broken all the way down. Arkansas keeps aged and physically-disabled home care in fee-for-service — you enroll once with the state's fiscal agent (Gainwell) and bill the state, rather than chasing contracts with several managed-care plans. PASSE managed care is a separate world that only touches behavioral-health and developmental-disability members, so as a personal-care agency you can ignore it. Do these steps in order:

- 1. Hold your ADH Private Care Agency license first.** Gainwell will ask for your license as part of provider enrollment — you can't enroll as a personal-care provider without it, so finish Part 4 before you start here.
- 2. Get your Type 2 NPI and pick the right taxonomy.** Your organizational NPI (Part 3) and a home-care/personal-care taxonomy are what Arkansas Medicaid uses to identify you on every claim.
- 3. Apply through the Gainwell provider portal.** Create your account and submit the enrollment application at portal.mmis.arkansas.gov, attaching your license, NPI, ownership disclosures, and W-9. Gainwell's Provider Assistance Center (800-457-4454) walks you through gaps — call them early rather than guessing.
- 4. Complete your AuthentiCare EVV enrollment.** Personal Care (and self-directed IndependentChoices) requires Electronic Visit Verification through AuthentiCare — set up your agency, caregivers, clients, and authorizations before your first billable visit, because an unverified visit doesn't get paid.
- 5. Take referrals through DHS assessment and case management.** Personal Care and ARChoices clients are authorized after a DHS/DAABHS assessment (the Arkansas Independent Assessment sets level of care); your service plan and delivered hours must match the authorization on file.
- 6. Bill T1019 against verified visits and reconcile.** Personal care bills under HCPCS T1019 (per 15 minutes) on the DHS fee schedule — pull the live rate, submit clean claims tied to AuthentiCare visits, and reconcile every remittance so nothing slips past the timely-filing window.

Why fee-for-service is actually good news for you. Because Arkansas pays personal care fee-for-service, you negotiate no rates and juggle no competing managed-care contracts — you enroll once, follow one fee schedule, and bill one payer. The trade-off is that the state pays on its own cycle, so keep private-pay clients and a payroll reserve to bridge the gap while Medicaid claims settle.

PART 9

What It Costs & How Long It Takes

In short: Budget the license, the \$1M liability policy, an RN, caregiver training, EVV, payroll float, and (optionally) accreditation. Launch on private pay, then add Medicaid.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
ADH license	Initial Private Care Agency license	Confirm fee with HFS
\$1M liability insurance	Required by the Private Care Agency rules	Annual premium
RN (supervision)	Employed or contracted RN	Ongoing
Business setup	SOS LLC \$45 + \$150/yr franchise tax, EIN free, NPI free, legal	~\$45 + \$150/yr + legal
Other insurance & bond	Abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Caregiver training	40-hr training + 12 hrs annual per caregiver	Per caregiver
EVV & software	AuthentiCare + scheduling/billing/payroll platform	Monthly SaaS
Payroll float	You pay caregivers before Medicaid/insurers pay you	Working capital

The cash-flow trap to plan for. You pay caregivers weekly, but Medicaid and insurers pay weeks later. Private-pay clients smooth this out — line up a business line of credit before you scale Medicaid hours.

9.2 Timeline

Weeks	Entity, RN & insurance	Form the LLC, line up your RN, bind the \$1M policy
Weeks	Policies	Write your P&P to the Private Care Agency rules
Submit	ADH application	Application to Health Facility Services; pass the pre-license inspection
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Parallel	Enroll for Medicaid	Gainwell enrollment + AuthentiCare EVV
Ongoing	Referrals & census	Discharge planners, AAAs, DAABHS case managers

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$26/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that sink underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay caregivers every week or two, but Arkansas Medicaid pays weeks after you bill a verified visit. On a growing Medicaid caseload you can be out thousands of dollars in wages you've paid but not yet collected. Keep several weeks of payroll in reserve, or lean on private pay (which pays up front) while your Medicaid line ramps.
- **Unbillable overhead:** your own time, a scheduler/coordinator, office and phone, background checks, and the RN supervisory visits the rules require — none of it is billable, all of it is real.
- **Turnover churn:** every caregiver who quits costs you re-hiring, re-screening (ASP/FBI, registries), and re-training the 40 hours. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Denied and delayed claims:** a Medicaid claim that doesn't match a verified AuthentiCare visit, or misses timely filing, simply doesn't get paid. Reconcile every remittance and chase exceptions weekly.
- **Annual renewals:** the ADH license renews each December 31, the \$150 franchise tax is due each May 1, and caregivers need 12 hours of in-service every year — calendar them all so nothing lapses.

PART 10

Getting Your First Clients

In short: Clients come from discharge planners, home-health/hospice partners, the 8 Area Agencies on Aging, the Choices in Living Resource Center, care managers, and senior communities. Build these before you open.

10.1 The referral sources

- **Hospital discharge planners & case managers:** the highest-volume source for post-hospital home support — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled hours their benefit doesn't cover; build reciprocal partnerships.
- **Area Agencies on Aging (8 in AR):** the regional AAAs (NW Arkansas, White River, East Arkansas, Southeast, CareLink/Central, West Central, Southwest, Western Arkansas) and their case managers are a primary pipeline for the waiver population — get listed and known.
- **Choices in Living Resource Center (the ADRC):** Arkansas's statewide aging information-and-referral line (1-866-801-3435) routes families to services — get on their radar.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities refer supplemental one-on-one care.
- **Note on 2-1-1:** Arkansas 2-1-1 is not fully statewide (it primarily serves NW Arkansas) — don't rely on it as a statewide channel; lean on the AAAs and Choices in Living instead.

10.2 What to ask each referral source

"How do you decide which home-care agency to refer a family to?"

"What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?"

"Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?"

"How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?"

"What makes an agency easy to work with, and what gets one dropped from your list?"

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
ADH — Health Facility Services	501-661-2201 · 1-800-223-0340 · adh.hfs@arkansas.gov	The Private Care Agency application & fee; the pre-license inspection; the \$1M insurance rule
Arkansas State Police — background checks	501-618-8000 · cbc.ark.org	The ASP criminal check + FBI fingerprint; the maltreatment registries
DHS — electronic background checks	humanservices.arkansas.gov	The registry checks and drug screen for Medicaid caregivers
Caregiver training resources	DHS provider training / your program	The 40-hr training + 12 hrs annual

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Arkansas Secretary of State	sos.arkansas.gov	Forming your LLC (\$45); the \$150 franchise tax
Arkansas DFA (ATAP)	atap.arkansas.gov	Employer withholding & unemployment insurance
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPES / CMS	nppes.cms.hhs.gov	Your Type 2 NPI (for Medicaid billing)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.2	The \$1M liability policy + abuse + non-owned auto + dishonesty bond + workers' comp

Funding, EVV & referrals

Who	Contact	What to verify / ask
Gainwell — provider enrollment	800-457-4454 · portal.mmis.arkansas.gov	Enrolling your NPI; the current Personal Care (T1019) rate; EVV enrollment
DHS/DAABHS — Personal Care & ARChoices	800-482-8988 (DMS) · 855-372-1084 (ARChoices)	Eligibility & authorizations; the fee-for-service model
Arkansas Medicaid — EVV / AuthentiCare	evvarkansas@dhs.arkansas.gov	AuthentiCare onboarding and compliance
VA Community Care / Optum (Region 3)	844-839-6120 · va.gov/communitycare	The VA Homemaker/HHA program; joining the CCN

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging (8)	agingarkansas.org	Local information & referral for older adults
Choices in Living Resource Center (ADRC)	1-866-801-3435	The statewide aging information & referral line
Hospital / SNF discharge planners	your local hospitals & SNFs	The highest-volume referral source
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; payer requirements

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I have to take Medicaid to run a Private Care Agency?

No. Many Arkansas agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without Gainwell enrollment and AuthentiCare EVV. You still need the ADH license and the \$1M policy to provide hands-on personal care, but you can add Medicaid once you have cash flow and a team.

Is Arkansas home care really run through PASSE managed care?

Not for your population. PASSE covers only behavioral-health and developmental-disability members. Aged and physically-disabled Personal Care and ARChoices are fee-for-service through DHS/DAABHS and Gainwell — so you enroll and bill the state's fiscal agent, not a managed-care plan.

Do I truly need the \$1,000,000 liability policy, or is that negotiable?

It's a firm requirement of the Rules for Private Care Agencies, so a licensed hands-on agency has to carry it — ADH looks for it before issuing the license. The good news is that home-care liability is comparatively inexpensive; an independent broker who writes this niche can usually meet the \$1M limit affordably. Bind it early so it's ready for your pre-license inspection.

Can I skip the ADH license if I only do companion and sitter work?

Possibly — if you provide strictly companion/sitter/homemaker help with no hands-on personal care and don't bill Medicaid, you generally fall outside the Private Care Agency license. But the moment you help with bathing, dressing, transferring, or any hands-on ADL, or you bill Medicaid, you need the license, the RN, and the \$1M policy. Confirm your exact scope with ADH before you rely on the exemption.

You can do this

Opening an Arkansas in-home care agency is very doable — a single Private Care Agency license, a real fee-for-service Medicaid market (Personal Care and ARChoices) alongside private pay, and low startup costs. Take it one rung at a time: form the business, bind the \$1M policy, line up your RN, get the ADH license, recruit and screen

great caregivers, launch on private pay, enroll with Arkansas Medicaid, and build referral relationships with the AAAs and Choices in Living. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in the Arkansas Rules for Private Care Agencies (2024), Act 853 of 2025, and current Arkansas Medicaid information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with ADH, Arkansas Medicaid, and qualified counsel before you rely on it. Confirm the current license fee, the Personal Care rate, and your EVV setup before you rely on them.